



EVERGREEN COMMUNITY CARE INC.

New Youth Worker Orientation Manual

Residential Care Services — British Columbia
Your complete guide to your first 90 days

Document Reference	ECC-ORI-2026-001
Version	1.0 — July 2026
Applies To	All new Youth Care Workers and Residential Support Workers
Approved By	Executive Director, Evergreen Community Care Inc.
Review Date	July 2027
Related Documents	Residential Care Program Manual (ECC-RC-POL-2026-003); HR Policy Binder (ECC-HR-POL-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002)

Welcome to Evergreen Community Care. This manual is your guide to everything you need to know in your first 90 days. Read it carefully, keep it with you, and refer to it throughout your orientation. When you're unsure, ask.



WELCOME TO EVERGREEN COMMUNITY CARE

You've chosen work that is genuinely important. The young people in our care have experienced significant adversity. Your presence — consistent, caring, and professional — will matter more to them than any program, policy, or procedure in this manual.

This orientation manual will walk you through what you need to know, what you need to do, and how you need to do it — in your first shift, your first week, your first month, and through your first 90 days. It is organized around three things:

- What you must know (policy, legislation, rights, ethics)
- What you must do (required training, tasks, and timelines)
- What good practice looks like (the skills, judgments, and habits that distinguish an excellent youth worker)



A note on online learning: Throughout this manual you'll see blue boxes like the ones below. These highlight specific online training modules, videos, and resources you can complete on your own schedule. Online learning is a supplement to — not a replacement for — supervised practice, mentorship, and team training.

Your orientation is not something that happens to you in your first week and then ends. It's a structured 90-day journey. You will have a mentor, you will have bi-weekly supervision, and you will not be left on your own before you're ready.

— Melvin Solomon M.S., Founder and Chief Executive Officer, Evergreen Community Care Inc.



TABLE OF CONTENTS

- PART 1** Your First 90 Days — Timeline and Milestones
- PART 2** Understanding Your Role
- PART 3** The Young People in Our Care
- PART 4** Rights, Law, and Your Legal Obligations
- PART 5** Safe, Therapeutic Practice
- PART 6** Documentation and Reporting
- PART 7** Medication, Health, and Safety
- PART 8** Cultural Safety and Equity
- PART 9** Mandatory Training Guide
- PART 10** Self-Care, Supervision, and Professional Growth
 - Appendix A Day 1 Orientation Checklist
 - Appendix B Week 1 Task Tracker
 - Appendix C 90-Day Milestone Sign-Off Sheet
 - Appendix D Online Learning Completion Tracker
 - Appendix E Key Contacts and Emergency Numbers
 - Appendix F Glossary of Key Terms



PART 1 — YOUR FIRST 90 DAYS: TIMELINE AND MILESTONES

YOUR ORIENTATION AT A GLANCE

Your first 90 days are structured into four phases. Each phase has specific requirements, tasks, and sign-offs. Your mentor and supervisor will support you through each phase.

PHASE	REQUIREMENTS AND MILESTONES
BEFORE DAY 1	✓ CRC Vulnerable Sector clearance received ✓ Offer letter signed and on file ✓ Policy acknowledgements signed ✓ Tax and payroll forms completed ✓ Schedule confirmed ✓ This manual received and reading begun
DAY 1	✓ Orientation with Program Manager (3–4 hours) ✓ Facility tour; team introductions; emergency procedures shown ✓ Keys, access, and IT systems set up ✓ WHMIS orientation completed ✓ OHS rights and hazard orientation completed ✓ Mentor assigned; first mentored shift begins today
WEEK 1	✓ First Aid / CPR Level C confirmed current ✓ Minimum 3 fully supervised shifts with mentor ✓ Program Manual read: Parts 1–5 ✓ Rights and mandatory reporting online module completed ✓ Shift log written under supervision ✓ First bi-weekly supervision meeting with Program Manager
WEEK 2	✓ Medication Administration Module 1 completed (if applicable) ✓ Program Manual read: Parts 6–10 ✓ First unsupervised shift completed ✓ NVCI Module 1 booked or completed ✓ 30-day check-in scheduled
30 DAYS	✓ 30-day check-in with Program Manager completed and signed ✓ WHMIS online module completed ✓ Privacy and FOIPPA online module completed ✓ Duty to Report online module completed ✓ Abuse Recognition and Prevention module completed ✓ Performance strengths and growth areas identified in writing
60 DAYS	✓ Indigenous Cultural Safety and Humility training completed ✓ Gender-Affirming Care training completed ✓ Trauma-Informed Practice foundations completed ✓ Suicide and Self-Harm Response (ASIST or equivalent) booked ✓ Second bi-weekly supervision completed ✓ Medication Administration competency confirmed (if applicable)
90 DAYS	✓ NVCI fully certified (all modules) ✓ ASIST or equivalent completed ✓ All mandatory training in Appendix D marked complete ✓ 90-day formal performance review completed and signed ✓ Probation status confirmed ✓ Appendix C signed by staff member and Program Manager ✓ Moving to monthly supervision cycle



Reminder: Training costs are covered by Evergreen Community Care. You are paid at your regular rate for all required training time, including online modules completed outside your shift.



1.1 Day 1 — Hour by Hour

Your first day is structured. Here is what to expect:

TIME	ACTIVITY
8:30–9:00	Arrive; meet Program Manager; receive your keys, access codes, and ID
9:00–9:30	Facility tour: every room, all exits, fire extinguisher and assembly point locations, first aid kit, medication cabinet, staff workspace, emergency contacts board
9:30–10:30	Orientation to the Program Manual: mission, values, rights of youth, code of ethics — this is not a passive reading exercise; you will discuss each section with your Program Manager
10:30–11:00	IT, documentation systems, and timekeeping setup; shift log overview; how to read the youth's ISPs and BSPs
11:00–11:30	WHMIS orientation: hazardous materials in this facility, SDS binder location, PPE location and use
11:30–12:00	OHS orientation: your rights as a worker, how to report a hazard, emergency procedures, working alone check-in procedure
12:00–12:30	Lunch with the team; meet any other staff on shift
12:30–14:00	Mentored shift begins: shadow your assigned mentor through the afternoon shift; observe interactions with youth, shift handover, and documentation
14:00–15:00	Meet your mentor formally; review your schedule for the first two weeks; discuss questions from the morning
15:00–15:30	Program Manager debrief: first impressions, questions, anything that needs clarifying before you return



PART 2 — UNDERSTANDING YOUR ROLE

WHAT YOUTH WORKERS ACTUALLY DO

A Youth Care Worker at Evergreen Community Care is not a babysitter, a security guard, or a caseworker. You are a therapeutic caregiver. The relationship you build with each young person is itself the primary intervention — more powerful than any program or technique. Everything else in this manual supports that relationship.

2.1 Core Responsibilities

Responsibility	What This Means in Practice
Physical care	Three nutritious meals and two snacks daily; clean, comfortable sleeping environment; appropriate clothing; personal hygiene support where indicated in the care plan
Emotional support	Consistent, warm, non-judgmental presence; responding to distress with empathy rather than alarm; helping young people name and regulate their emotions
Safety	Active monitoring of the physical environment and all youth at all times; wellness checks as specified in each youth's care plan; immediate action on any safety concern
Therapeutic relationship	Reliable, boundaried, genuine connection; knowing each youth's history, triggers, strengths, and goals; responding to behaviour as communication
Rights protection	Actively upholding every right listed in CFCSA s.70; ensuring youth can access the Advocate, their social worker, and emergency contacts without restriction
Documentation	Accurate, objective shift log entries completed before you leave; medication records; incident reports when required
Education support	Supporting school attendance and homework; liaising with the school when problems arise; attending school meetings as part of the care team
Community inclusion	Facilitating community activities and outings; supporting cultural connection; building life skills every shift
Family connection	Supporting youth to maintain contact with family and community as specified in the care plan; never creating barriers to authorized contact

2.2 Professional Boundaries



Professional boundaries exist to protect young people — not to keep you at a distance. A boundary is not a wall; it's the framework that makes a safe therapeutic relationship possible. The following are absolute requirements:

- Never share personal contact information (personal phone number, social media, personal email) with a current youth
- Never maintain personal social media connections with current or recently discharged youth
- Never accept gifts of significant value from youth or their families
- Never discuss other youth's personal information with youth in your care
- Never use physical contact that is not clearly therapeutic, culturally safe, and welcomed by the youth — ask first, always
- Disclose any pre-existing personal relationship with a youth or family member to your supervisor immediately — before the placement proceeds
- If you find yourself thinking about a particular youth outside of work in ways that feel personal, bring it to supervision — this is secondary trauma, and it needs support, not silence

ONLINE LEARNING



Professional Boundaries in Residential Care

MCFD has developed an online module on professional boundaries specific to residential care settings. Ask your Program Manager for the current link. Expected completion: Before first solo shift. Certificate: Required.

2.3 What Excellent Youth Workers Have in Common

After 15 years of frontline and leadership experience in BC social services, our leadership has observed that the workers who make the greatest difference share a specific set of qualities. These are not personality traits you either have or don't — they are habits of mind that can be learned and practiced:

- Curiosity over judgment: they ask 'what happened to you?' before 'what's wrong with you?'
- Presence over procedure: they're genuinely there with the young person, not just completing tasks
- Consistency over charisma: they show up the same way, every shift, regardless of their mood
- Humility over confidence: they ask for help, bring cases to supervision, and say 'I don't know'
- Honesty over comfort: they write the difficult shift log entry, report the concerning incident, give the honest feedback
- Boundaries over connection: they maintain professional limits precisely because they care deeply

PART 3 — THE YOUNG PEOPLE IN OUR CARE

WHO THE YOUTH ARE AND HOW WE UNDERSTAND THEM

3.1 Understanding Trauma

Virtually every young person in government care has experienced significant trauma — abuse, neglect, family violence, loss, instability, or combinations of these. Trauma changes the brain, the nervous system, and how people experience and respond to relationships and stress.

Understanding this doesn't mean excusing harmful behaviour — it means understanding it accurately so you can respond effectively. When a youth screams at you, throws something, refuses to go to school, or shuts down completely, they are almost always communicating something they cannot express any other way. Your job is to understand the communication, respond to the need, and help them build a wider vocabulary for their inner life.

ONLINE LEARNING

Trauma-Informed Practice Foundations



WorkBC / MCFD-aligned online course: 'Introduction to Trauma-Informed Practice in Residential Care.' Search the BC Campus Open Campus platform or ask your Program Manager for the current approved module. 2–3 hours. Required within 60 days. Annual refresher required.

3.2 Attachment and Why It Matters

Many youth in care have disrupted, insecure, or traumatic attachment histories. They may have never had a reliable adult — or may have learned that adults who get close eventually leave, abuse, or abandon them. This shapes how they relate to you.

They may test your reliability — aggressively, repeatedly. They may push you away precisely when they most want connection. They may refuse to engage with the care plan because every previous plan failed. This is not defiance — it is survival logic, based on experience. Your consistent, boundaried, non-retaliating presence over time is literally therapeutic.

3.3 Indigenous Youth and Cultural Identity

Indigenous children and youth are vastly overrepresented in BC government care — a direct legacy of colonization, residential schools, and the Sixties Scoop. When you work with an Indigenous youth, you are working with someone who carries not only their personal history but a collective history of systemic harm in child welfare settings.

Cultural identity is not supplementary to care — for Indigenous youth, it is central to identity, healing, and self-determination. You are required to:

- Support every Indigenous youth's Cultural Plan actively — not just allow cultural activities to happen
- Facilitate access to Elders, cultural mentors, DAA representatives, and cultural practices
- Accommodate smudging, ceremony, traditional foods, and cultural dress without judgment
- Never make assumptions about cultural identity based on appearance or name
- Complete Indigenous Cultural Safety and Humility training within 60 days — this is mandatory, not optional

ONLINE LEARNING

Indigenous Cultural Safety and Humility



San'yas Indigenous Cultural Safety Training (BC health authorities standard): sanyas.ca — ask your Program Manager to arrange organizational access. Alternatively: FNHA Cultural Safety module via fnha.ca/wellness. 6–8 hours. Required within 60 days. Annual renewal required.

3.4 Gender Identity and Gender-Affirming Care

MCFD Standards (amended April 2024) require all residential care staff to provide gender-affirming care as standard practice. This is not optional.



MANDATORY: You must use every youth's correct name and pronouns from Day 1, consistently. Deliberate misgendering is a Code of Ethics violation subject to disciplinary action. If you are unsure of a youth's name or pronouns, ask. Ask respectfully. Ask privately.

Gender-affirming care in daily practice means:

- Using correct name and pronouns — always, consistently, including when the youth is not present
- Supporting access to gender-affirming clothing, personal care items, and hygiene products
- Treating each youth's gender identity as confidential — not shared with family or others without informed consent
- Creating a physically and emotionally safe environment for youth of all gender identities and expressions
- Responding immediately to any gender-based discrimination or harassment from staff, youth, or visitors

ONLINE LEARNING

Gender-Affirming Care Training



Rainbow Health Ontario / PFLAG Canada offer BC-accessible online training on gender-affirming care in institutional settings. Also: Trans Care BC's 'Gender-Affirming Care for Youth' training at phsa.ca. Check with your Program Manager for the current approved module. Required within 60 days. Annual renewal.

PART 4 — RIGHTS, LAW, AND YOUR LEGAL OBLIGATIONS

THE LEGAL FRAMEWORK YOU WORK WITHIN

You do not need to memorize every section of every Act. You do need to understand the key rights, the key obligations, and the key limits on your authority. Here is what you must know before your first solo shift.

4.1 The Rights of Every Youth in Care — CFCSA Section 70

These rights are legal entitlements. They cannot be withheld as consequences or granted as rewards. Every young person in our care is entitled to:

- Be fed, clothed, and nurtured according to community standards — the same quality of care as any child in the home
- Know their plan of care and be consulted on significant decisions affecting them
- Reasonable privacy and possession of their personal belongings
- Be free from corporal punishment — no physical punishment, ever, for any reason
- Know what behaviour is expected and what the consequences of not meeting those expectations will be
- Receive medical and dental care when required
- Participate in social and recreational activities appropriate to their age and abilities
- Religious instruction and participation in religious activities of their choice
- Guidance and encouragement to maintain their cultural heritage
- An interpreter if language or disability is a barrier
- Private discussion with family members, subject to court orders
- Private discussion with their lawyer, the Representative for Children and Youth, the Ombudsperson, an MLA, or an MP
- Information about and assistance contacting the RCY and Ombudsperson
- Know their rights and the procedures for enforcing them



MANDATORY: You must know these rights. You will be asked to explain them to new youth at admission. You will be assessed on your knowledge of them in your 90-day review.



ONLINE LEARNING

Rights of Children and Youth in Care — CFCSA

BC Government eLearning Hub (learning.gov.bc.ca): search 'Rights of Children and Youth in Care.' This module covers s.70 rights, complaint procedures, and the role of the RCY. Also available at rcybc.ca. 1 hour. Required within 7 days of hire.

4.2 Your Mandatory Duty to Report — CFCSA Section 14



MANDATORY: Section 14 of the CFCSA requires EVERY person — including you — to report to MCFD immediately if they have reasonable grounds to believe a child needs protection. This duty is personal, immediate, and non-delegable. You cannot fulfill it by telling your supervisor. You must report yourself.

You must report when you have reasonable grounds to believe a child:

- Has been or is likely to be physically harmed by a parent or caregiver
- Has been or is likely to be sexually abused or exploited
- Has been or is likely to be neglected
- Is emotionally harmed by the conduct of a parent or caregiver
- Is exposed to family violence

Reporting applies even if:

- The information was shared in confidence
- You reported a previous incident involving the same child
- You believe someone else has already reported
- The concern involves a colleague or supervisor

MCFD Provincial Centralized Screening (24 hours): 1-800-663-9122



ONLINE LEARNING

Duty to Report — Mandatory Reporting Under the CFCSA

BC government online module: 'Reporting Child Abuse and Neglect: It's Your Duty' — available at mcf.gov.bc.ca/reporting. Covers the legal duty, what to report, how to report, and what happens after. 90 minutes. Required within 30 days. Biennial renewal.

4.3 Prohibited Practices — What You Must Never Do

The following are prohibited under the CFCSA, MCFD Standards, and Evergreen Community Care policy. No context, no urgency, and no instruction from a supervisor makes any of these acceptable. If you are told to do any of these, refuse and report:

Prohibited Practice	Why It Is Prohibited
Corporal punishment (any physical force as a consequence)	CFCSA s.70; MCFD Standards C.6 — absolute prohibition
Seclusion or confinement (preventing a youth from leaving any space)	MCFD Policy 5.13; CFCSA s.70 — abuse
Withholding meals, bedding, or any basic physical need	CFCSA s.70 — removes a legal right

Humiliation, ridicule, or shaming	MCFD Standards D.7 — psychological harm
Unauthorized restriction of family contact	CFCSA s.70 — removes a legal right
Threatening removal or placement change to control behaviour	MCFD Standards — coercive and harmful
Physical restraint used as punishment or deterrent	MCFD Policy 5.13 — abuse
Any restraint technique not covered in NVCI certification	OHS Reg.; MCFD Policy 5.13
Prone (face-down) restraint	MCFD Policy 5.13 — risk of asphyxiation; absolute prohibition
Disclosing a youth's care status, identity, or information to unauthorized persons	FOIPPA; CFCSA confidentiality requirements

4.4 Privacy and Confidentiality — FOIPPA

Every young person's personal information — their name, their care status, their history, their diagnoses, their family, their gender identity — is confidential. You handle this information only for the purpose of providing care, and you share it only with those who need it for that purpose.

- Never discuss a youth's personal information on social media — not even in vague terms that could identify them
- Never photograph a youth without written MCFD authorization
- Never confirm or deny to any unauthorized person that a youth is in government care
- When police, media, or members of the public ask, refer them to the Program Manager immediately

ONLINE LEARNING

Privacy and Confidentiality — FOIPPA



BC OIPC (Office of the Information and Privacy Commissioner): oipc.bc.ca — 'Understanding FOIPPA' module. Also: MCFD online privacy training module available through your Program Manager. 1 hour. Required within 30 days. Annual renewal.



PART 5 — SAFE, THERAPEUTIC PRACTICE

DAILY PRACTICE THAT MAKES A DIFFERENCE

5.1 The Shift — Structure, Safety, and Presence

Every shift has the same basic structure. Learn it until it's automatic.

SHIFT MOMENT	WHAT YOU DO
Arrival	Sign in; confirm you have your working-alone device; check the board for any urgent updates before the outgoing shift leaves
Handover	15-minute minimum verbal handover from the outgoing shift lead: location and status of every youth; mood, presentation, incidents from the last shift; medications given; outstanding tasks; controlled substance count reconciled
Safety check	Brief walkthrough: all exits clear; medication cabinet locked; chemical storage locked; all youth located and accounted for; no new environmental hazards
During the shift	Be present with youth: meals together, activities, school support, community time, one-on-one connection; document significant moments as they happen — don't rely on memory at shift end
Wellness checks	As specified in each youth's ISP — minimum every 30 minutes for youth with safety plans; minimum hourly for all others; document every check during overnight shifts
Incidents	If anything significant happens: stay calm; ensure safety; call supervisor; document immediately; follow the relevant protocol (self-harm, missing youth, restraint, medical emergency — see Part 6)
Shift log	Complete a shift log entry for every youth before you leave. This is not optional and it cannot be delegated. Sign and date every entry. See Part 6 for standards.
Departure	Handover to incoming shift; confirm all tasks transferred; check in with your working-alone contact if on a solo shift; sign out

5.2 De-escalation — Your Most Important Clinical Skill

De-escalation is not a personality trait — it is a learnable skill set. The goal is not to 'win' a confrontation or even to stop a behaviour; it is to help a regulated nervous system return to a state where the young person can think and feel and connect. Here is the sequence:

STAGE	SIGNS	YOUR RESPONSE
Baseline (regulated)	Calm, engaged, within normal range for this youth	Relationship time; positive reinforcement; proactive BSP

		review
Agitation (early triggers)	Restlessness; pacing; voice change; shorter responses; avoiding	Reduce demands; offer a quiet choice; create space; use BSP strategies
Escalation (heightening)	Verbal aggression; property interference; threatening posture	Calm, low voice; do not engage the content of threats; call for backup; clear area for other youth
Crisis (active danger)	Physical aggression; active self-harm; immediate safety risk	Safety first; backup on scene before intervening physically; NVCI holds if threshold met and you are certified; call 911 if needed
De-escalation (coming down)	Reducing aggression; beginning to respond to calm voice	Maintain quiet; do not debrief immediately; give physical and emotional space
Recovery (calm restored)	Youth is regulated and accessible	Welfare check; quiet reconnection; debrief when the youth is ready; document thoroughly

ONLINE LEARNING

Non-Violent Crisis Intervention (NVCI) — Online Pre-Work



CPI (Crisis Prevention Institute): [crisisprevention.com](https://www.crisisprevention.com) — Review the NVCI Workbook online (provided by your employer). Module 1 of the NVCI certification covers verbal de-escalation and can be completed online before your in-person training day. Required within 90 days; Module 1 before first solo shift.

5.3 Behaviour Support Plans — How to Read and Use Them

Every youth with a history of significant behaviour has a Behaviour Support Plan (BSP) in their ISP. Before working with any youth, read their BSP. It tells you:

- Their known triggers — the environmental, relational, and situational factors most likely to lead to escalation
- Their early warning signs — what their escalation looks like at the beginning, when you can still redirect it
- Specific de-escalation strategies that work for this youth — these are individualized and different for every person
- What NOT to do — approaches that make things worse for this youth specifically
- Post-crisis recovery strategies — what this youth needs after a difficult episode



Practice tip: Read every BSP before you work with a youth for the first time. Read it again after any incident. Bring questions about a BSP to supervision. A BSP is only useful if it's actually guiding your moment-to-moment decisions.



5.4 Physical Restraint — Last Resort Only



MANDATORY: Physical restraint may only be used when there is an IMMINENT risk of serious physical harm, all less-restrictive approaches have failed, you are currently NVCI-certified, and a second staff member is present or has been called. It is never a consequence, deterrent, or management tool. Every use is a Critical Incident.

Prone (face-down) restraint is prohibited under all circumstances. Any hold restricting breathing or blood flow is prohibited under all circumstances. Seclusion is prohibited under all circumstances.

After any use of physical restraint, you must: conduct a medical wellness check immediately; notify your supervisor; debrief with the youth when regulated; offer EFAP access to all involved staff; complete a Critical Incident Report within 2 hours; notify the MCFD social worker.



PART 6 — DOCUMENTATION AND REPORTING

IF YOU DIDN'T WRITE IT DOWN, IT DIDN'T HAPPEN

Documentation in residential care is not paperwork for its own sake. The shift log is a legal document. Critical Incident Reports are reviewed by MCFD. Everything you write could be read by a court, a licensing officer, or a lawyer. Everything you fail to write could be equally significant.

6.1 Shift Log Standards

Every shift log entry must be:

- Accurate — what actually happened, observed, or was said; not assumptions or inferences
- Objective — factual, specific, and observable language; not labels, not interpretation
- Contemporaneous — written before you leave the facility; never reconstructed the next day
- Complete — significant events are not omitted because they reflect badly on you or a colleague
- Professional — no derogatory language, no diagnostic labels not in the youth's formal record

Minimum required entries per shift, per youth:

- Location and status at start and end of shift
- General mood and presentation (objective description)
- Meals — what, how much, any refusals or concerns
- School attendance or educational activity
- Medications — reference to MAR; any exceptions noted
- Any appointments attended
- Community activities or outings
- Family or significant person contact
- Any behaviour of note — positive or challenging — with specific factual description
- Any incident — even if handled smoothly — with reference to Incident Report if completed
- Outstanding tasks for the incoming shift
- Your name, title, and signature

The difference between a good log entry and a poor one:

POOR — Do not write this	BETTER — Write this instead
'Youth was difficult today and refused to co-operate.'	'At 14:30, youth refused to participate in the planned community outing, stating she did not want to go. Staff offered an alternative indoor activity; youth declined. Youth spent the afternoon in her room. At 17:00, youth came downstairs and joined the group for dinner. Mood appeared improved by end of shift.'

'Youth was aggressive.'	'At 19:15, youth threw a plastic cup at staff member [name] during a limit-setting conversation about phone use. No injury resulted. Staff used verbal de-escalation per BSP; youth returned to their room. Supervisor notified at 19:20.'
'Good shift. Youth okay.'	'Youth [name] was calm and engaged throughout the shift. Attended school (confirmed with school office), completed homework at 16:30, assisted with dinner preparation, and asked to call his aunt at 19:00. Call made; youth was in positive spirits afterward. Wellness checks completed: 23:00, 00:30, 02:00 — youth sleeping.'

6.2 Critical Incident Reporting

The following must be reported to your supervisor immediately and to MCFD within the timelines below:

CATEGORY	WHAT TRIGGERS IT	TIMELINE
Category 1 — Same Day	Death; hospitalization; abuse/neglect allegation; suicidal crisis or attempt; self-harm with injury; missing youth; police contact; physical restraint; traumatic exposure; school suspension	Verbal to MCFD within 2 hours; written CI Report within 24 hours
Category 2 — Next Business Day	Marked behaviour change; medication error with medical attention; property damage over \$500; youth criminal activity; unauthorized overnight	Written CI Report by end of next business day
Category 3 — Weekly	Minor incidents with no immediate safety concern; pattern concerns	Included in next weekly report to MCFD social worker



MANDATORY: If MCFD social worker is unavailable for a Category 1 incident: call Provincial Centralized Screening immediately. PCS 24-hour line: 1-800-663-9122.



ONLINE LEARNING

Critical Incident Reporting — Practice Module

MCFD provides online modules on critical incident reporting requirements for residential care providers. Ask your Program Manager for the current access link. 1 hour. Review this before your first solo shift.

PART 7 — MEDICATION, HEALTH, AND SAFETY

HEALTH CARE AND OCCUPATIONAL SAFETY

7.1 Medication Administration — The Five Rights



MANDATORY: You may NOT administer any medication until you have completed Medication Administration training and your authorization has been recorded in writing by the Program Manager. No exceptions.

When you are authorized, every medication administration follows the Five Rights — verified before every single dose:

- Right YOUTH — confirm identity before preparing anything
- Right MEDICATION — name and strength match the MAR exactly
- Right DOSE — the prescribed dose; never assume it's the same as last time
- Right ROUTE — oral, topical, inhaled — confirm the method
- Right TIME — within the prescribed time window; document the actual time administered

Observe the youth taking the medication before completing the MAR entry. Never pre-sign. Never guess. Any error — wrong drug, wrong dose, wrong youth — is an immediate medical situation and a Critical Incident.

ONLINE LEARNING



Medication Administration — Online Pre-Work

BC College of Nurses and Midwives publishes medication administration practice standards online. Also: 'Safe Medication Administration in Community Care' — available through the BC Medication Administration curriculum your employer will provide. Module 1 can be completed online before in-person competency demonstration. Required if your role includes medication duties.

7.2 Your OHS Rights as a Worker

The Workers Compensation Act gives you three fundamental rights:

- The right to KNOW about hazards in your workplace
- The right to PARTICIPATE in health and safety decisions through the Joint Health and Safety Committee
- The right to REFUSE work you reasonably believe poses an undue hazard — without retaliation

You will receive a full OHS orientation on Day 1. Your OHS Program Binder (ECC-OHS-POL-2026-002) is kept in the program office and available to you at all times.

7.3 Workplace Violence and Your Safety

Working with children and youth who have experienced significant trauma means working in a higher-risk environment for workplace violence. This is acknowledged, planned for, and supported.

- De-escalation is always the first response — NVCI gives you the skills to do this
- You are never required to physically manage a behavioural crisis alone — call for backup before escalating to physical intervention
- All physical assaults on staff — including assaults with no injury — are reported on the Incident Report form within 2 hours
- After a violent incident, you will be offered EFAP access, a debrief with your supervisor, and modified duties if needed
- You will never be penalized for reporting a violent incident or for refusing an unsafe task

7.4 Working Alone — Overnight and Solo Shifts

If you work solo overnight shifts, you are in a working-alone situation under the OHS Regulation. You must:

- Confirm your working-alone check-in device (phone or personal alarm) is charged and functional at the start of every solo shift
- Check in with the designated contact person at the required intervals — typically every 2 hours for overnight shifts
- Know the escalation protocol if you miss a check-in — your contact will try to reach you, then call 911 if unsuccessful
- Complete working-alone safety training before your first solo overnight shift

ONLINE LEARNING



Working Alone Safety

WorkSafeBC provides a free online working-alone training module at worksafebc.com. Search 'Working Alone.' 30 minutes. Required before first solo overnight shift.

PART 8 — CULTURAL SAFETY AND EQUITY

PRACTICE THAT AFFIRMS EVERY YOUNG PERSON'S IDENTITY

8.1 What Cultural Safety Means in Practice

Cultural safety is not a training event — it is a daily orientation. It means creating an environment where every young person's cultural identity is affirmed, supported, and treated as a source of strength rather than difference or difficulty.

Cultural safety is especially critical for Indigenous youth, whose cultural identity has been actively suppressed by Canada's child welfare system for generations. For every other young person, it means:

- Incorporating cultural foods, celebrations, and practices into daily life
- Ensuring the physical environment reflects the cultural diversity of youth in residence — art, images, books, media
- Challenging stereotypes, assumptions, and discriminatory comments from any person in the program
- Examining your own cultural assumptions and how they show up in your practice — in supervision, not just in training

8.2 Anti-Racism as Active Practice

Racism in residential care is not only overt discrimination — it is also the accumulated weight of policies, practices, and interactions that treat racialized youth as less than. Your role:

- Model respectful, anti-racist behaviour in every interaction
- Respond directly and immediately to any racist comment, joke, or behaviour from any person in the program — silence is complicity
- Bring your own bias to supervision and to training — cultural humility is not achieved; it is practised
- Report experiences of racism that youth report to you as a Critical Incident; validate the youth's experience first

ONLINE LEARNING

Anti-Racism and Cultural Humility in Practice



FNHA 'Indigenous Cultural Humility' self-paced course: fnha.ca. BC ANTI-RACISM Data Act resources via gov.bc.ca. Also: 'Dismantling Anti-Indigenous Racism' (UBC MOOC, available via edX). Discuss with your Program Manager which module best suits your role.

PART 9 — MANDATORY TRAINING GUIDE

EVERY REQUIRED TRAINING, IN ONE PLACE

All mandatory training is tracked centrally by Human Resources. Your training record is reviewed at your 30-, 90-, and 180-day check-ins and at your annual performance review. Staff with overdue mandatory training are removed from duties requiring that training until compliance is restored.

TRAINING	FORMAT + PROVIDER	TIMELINE	RENEWAL
OHS Orientation + Program Manual review	In-person with Program Manager	Day 1	At each annual update
Youth Rights (CFCSA s.70)	Online: bcgov eLearning hub + in-person discussion	Before first solo shift	Every 2 years
Mandatory Reporting (CFCSA s.14)	Online: mcf.gov.bc.ca/reporting (90 min)	Within 30 days	Every 2 years
Privacy and FOIPPA	Online: oipc.bc.ca module (60 min)	Within 30 days	Annual
WHMIS 2015	Online: worksafebc.com + facility walkthrough	Before first shift	Every 3 years
Abuse Recognition and Prevention	Online module (provider via Program Manager)	Within 30 days	Annual
Professional Boundaries	Online module (MCFD-aligned)	Before first solo shift	Annual
First Aid / CPR Level C	In-person certification	Within 30 days	Per certification cycle
Non-Violent Crisis Intervention (NVCi)	In-person certification — CPI or approved equivalent	Module 1 before solo shift; full cert within 90 days	Annual recertification
Trauma-Informed Practice	Online: BC Campus or approved module (2–3 hrs)	Within 60 days	Every 2 years
Indigenous Cultural Safety and Humility	Online: San'yas (sanyas.ca) or FNHA (6–8 hrs)	Within 60 days	Annual
Gender-Affirming Care	Online: Trans Care BC or PFLAG module	Within 60 days	Annual
Suicide and Self-Harm Response (ASIST or SafeTALK)	In-person certification — LivingWorks	Within 60 days	Annual (SafeTALK) / Biennial (ASIST)
Medication Administration	Online pre-work + in-person competency	Before any medication duty	Annual competency



Working Alone Safety	Online: worksafebc.com (30 min)	Before first solo overnight	Annual
Driver Safety (with abstract)	Employer orientation + abstract on file	Before driving youth	Annual abstract review

9.1 Online Learning Platform Guide

The following platforms are approved sources for mandatory online training modules. All are free or employer-provided:

Platform / Provider	Modules Available
worksafebc.com	WHMIS 2015; Working Alone; Workplace Violence Prevention; Young and New Worker; Right to Refuse Unsafe Work
BC Government eLearning Hub (learning.gov.bc.ca)	Rights of Children and Youth in Care; FOIPPA basics; MCFD Standards overview; Accessibility
mcf.gov.bc.ca	Mandatory Reporting; Child Welfare Policy modules; Indigenous child and family services
San'yas Indigenous Cultural Safety (sanyas.ca)	Indigenous Cultural Safety and Humility — Level 1 and 2; specific modules for social services
FNHA (fnha.ca)	Cultural Safety; FNHA wellness resources; Indigenous health frameworks
Trans Care BC / PHSA (phsa.ca)	Gender-Affirming Care for Youth; pronoun use; clinical considerations
BC Campus (bccampus.ca/open-textbooks)	Trauma-Informed Practice; Indigenous studies; community care practice
LivingWorks (livingworks.net)	ASIST (Applied Suicide Intervention Skills Training); safeTALK (half-day)
Crisis Prevention Institute (crisisprevention.com)	NVCI online pre-work; verbal de-escalation resources
PFLAG Canada (pflagcanada.ca)	Gender identity and sexual orientation resources; supporting LGBTQ2S+ youth



Online Learning Tip: Complete online modules when you are alert and uninterrupted — not at the end of an overnight shift. Take notes as you go. Bring your questions to your next supervision session. The goal is genuine understanding, not a completed checkbox.



PART 10 — SELF-CARE, SUPERVISION, AND PROFESSIONAL GROWTH

TAKING CARE OF YOURSELF SO YOU CAN TAKE CARE OF OTHERS

10.1 Secondary Traumatic Stress and Compassion Fatigue

Working with young people who have experienced significant trauma puts you at genuine risk of secondary traumatic stress (STS) and compassion fatigue. These are recognized occupational health concerns — not signs of weakness or unsuitability for the work.

STS can look like: intrusive thoughts or dreams about youth's disclosures; emotional numbness or detachment; heightened anxiety or hypervigilance outside of work; declining satisfaction with your work; physical symptoms including sleep disturbance, fatigue, or appetite changes.

If you notice any of these signs in yourself, bring them to supervision. This is what supervision is for. Early support is dramatically more effective than waiting until you're in crisis.

ONLINE LEARNING



Secondary Traumatic Stress — Self-Assessment and Support

ProQOL (Professional Quality of Life Scale): proqol.org — free, validated self-assessment tool for STS and compassion fatigue. 10 minutes. Take it at 30 days and 90 days. Bring the results to supervision. Also: 'Preventing and Healing from Vicarious Trauma' — BC EFAP resources.

10.2 Your Supervision — What to Expect and How to Use It

During your first 90 days, you have bi-weekly supervision with your Program Manager. Supervision is not a performance evaluation (though your manager does observe and provide feedback). It is a protected space for reflection, learning, and support.

Come to every supervision session with:

- At least one situation from the past two weeks that was challenging — be specific
- At least one situation that went well — what did you do, and why did it work?
- Any questions that came up during the shift that you weren't sure how to handle
- An honest answer to: how am I doing emotionally in this work?



Supervision is confidential. Notes from supervision sessions are kept in your personnel file and accessible only to you and your supervisor. You will not be penalized for honesty in supervision. The whole point is that you can say the difficult things here.

10.3 Employee and Family Assistance Program (EFAP)



You have access to Evergreen Community Care's EFAP from your first day of employment. EFAP provides:

- Up to 8 free, confidential counselling sessions per year — for you and eligible family members
- Financial counselling, legal consultation, and life coaching
- 24-hour crisis phone line

The EFAP number is: [posted in the staff area]. Your use of EFAP is completely confidential. Evergreen Community Care receives only aggregate utilization data — never individual information.

10.4 Your Professional Development Plan

At your 90-day review, you and your Program Manager will develop a Professional Development Plan for your first full year. This may include:

- Specialized training in areas relevant to the current youth population (e.g., eating disorder support, substance use, developmental disability)
- Clinical supervision with a registered professional
- Leadership or shift lead development opportunities
- Pursuit of further credentials in Child and Youth Care or a related field
- Mentorship or peer learning partnerships



Career growth at Evergreen: We promote from within wherever possible. The path from support worker to shift lead to program manager to executive is one that members of our leadership team have walked. If this work is where you want to build your career, tell your supervisor. We will invest in that.



Appendix A. DAY 1 ORIENTATION CHECKLIST

Complete this checklist on Day 1. Both the new worker and the Program Manager sign at the bottom. File in the personnel file.

Staff Name: _____ Date: _____ Position: _____

☐	ORIENTATION TOPIC
☐	Received and signed the Offer Letter, Code of Ethics, and Policy Acknowledgements
☐	Criminal Record Check — Vulnerable Sector clearance confirmed on file
☐	Keys, facility access codes, and IT login set up and tested
☐	Photo ID issued or recorded
☐	Facility tour: all rooms, exits, fire extinguisher locations, first aid kit, AED (if present), assembly point
☐	Emergency contacts board shown and explained: 911, MCFD PCS, on-call supervisor, Poison Control, Advocate, Crisis Line
☐	Medication cabinet location and locking procedure shown
☐	Chemical storage (WHMIS) location and SDS binder shown
☐	PPE location (gloves, masks, eye protection) shown
☐	Working-alone check-in device and procedure explained
☐	OHS rights explained: right to know, right to participate, right to refuse unsafe work
☐	Hazard reporting procedure explained
☐	Emergency evacuation procedure practiced (verbal walkthrough)
☐	Program Manual provided; Parts 1–3 reviewed with Program Manager
☐	Rights of Youth in Care (CFCSA s.70) explained
☐	Mandatory Reporting duty (CFCSA s.14) explained — personal, immediate, non-delegable
☐	Code of Ethics and professional boundaries discussed
☐	Prohibited practices list reviewed
☐	Shift log and documentation system explained; sample entry reviewed



☐	ISP and BSP format and access explained
☐	Medication Administration: authorization status confirmed; if authorized, process reviewed
☐	MCFD contact information: social workers, PCS after-hours number (1-800-663-9122)
☐	EFAP access information provided
☐	Mentor assigned; first mentored shift begins today
☐	Schedule confirmed for first two weeks
☐	Questions answered and documented below

Questions raised by new staff member:

Staff Signature / Date: _____

Program Manager Signature / Date: _____



Appendix B. WEEK 1 TASK TRACKER

Complete by end of Week 1. Check off each task. Bring this to your first bi-weekly supervision session.

☐	TASK	COMPLETED
☐	Day 1 Orientation Checklist completed and signed	
☐	Minimum 3 supervised mentored shifts completed	
☐	First aid / CPR Level C certificate confirmed current and on file	
☐	Read Program Manual Parts 1–5 (cover through Part 5)	
☐	Online module: Youth Rights and Mandatory Reporting (bcgov eLearning)	
☐	Online module: WHMIS 2015 (worksafebc.com)	
☐	Online module: Working Alone Safety (worksafebc.com) — if applicable to role	
☐	BSPs for all currently placed youth read and reviewed with mentor	
☐	Written a complete shift log entry under supervision; reviewed by mentor	
☐	On-call supervisor schedule confirmed and number saved	
☐	MCFD social worker contact information for all youth saved	
☐	First bi-weekly supervision meeting with Program Manager scheduled	
☐	NVCI Module 1 booked or completed	
☐	Medication Administration training booked (if your role includes this)	

Mentor Name: _____ Staff Signature / Date: _____



Appendix C. 90-DAY MILESTONE SIGN-OFF SHEET

Completed at the 90-day formal performance review. Both parties sign. Copy to personnel file.

Staff Name: _____ Start Date: _____ Review Date: _____

MILESTONE	COMPLETED DATE	VERIFIED BY
All mandatory training listed in Appendix D completed		
First Aid / CPR Level C — certificate on file		
NVCI full certification — certificate on file		
ASIST or SafeTALK — certificate on file		
Indigenous Cultural Safety and Humility — certificate on file		
Gender-Affirming Care — certificate on file		
Trauma-Informed Practice — completion confirmed		
Medication Administration authorization confirmed (if applicable)		
30-day check-in completed and signed		
60-day check-in completed and signed		
Program Manual read and discussed in full		
90-day formal performance review completed		
Professional Development Plan for Year 1 agreed		
Probationary status confirmed		



Program Manager Assessment (narrative — attach if needed):

Strengths demonstrated during orientation period:

Areas for continued development:

Staff Signature / Date: _____ Program Manager Signature / Date: _____

**Appendix D. ONLINE LEARNING COMPLETION TRACKER**

Complete the details as you finish each online module. Attach any certificates to this sheet. Submit to HR upon completion of each module.

MODULE	PLATFORM / PROVIDER	DATE COMPLETED	CERT. ATTACHED?
Youth Rights — CFCSA s.70	BC Government eLearning Hub		☐ Yes ☐ No
Mandatory Reporting — CFCSA s.14	mcf.gov.bc.ca		☐ Yes ☐ No
Privacy and FOIPPA	oipc.bc.ca module		☐ Yes ☐ No
WHMIS 2015	worksafebc.com		☐ Yes ☐ No
Abuse Recognition and Prevention	Provider: [as assigned]		☐ Yes ☐ No
Professional Boundaries	MCFD-aligned module		☐ Yes ☐ No
Trauma-Informed Practice	BC Campus / approved provider		☐ Yes ☐ No
Indigenous Cultural Safety and Humility	San'yas (sanyas.ca) or FNHA		☐ Yes ☐ No
Gender-Affirming Care	Trans Care BC / PHSA		☐ Yes ☐ No
NVCI Pre-work (Module 1 online)	crisisprevention.com		☐ Yes ☐ No
Suicide and Self-Harm Response (pre-work)	livingworks.net / ASIST pre-reading		☐ Yes ☐ No
Working Alone Safety	worksafebc.com		☐ Yes ☐ No
ProQOL Self-Assessment (30 days)	proqol.org		N/A — bring results to supervision
ProQOL Self-Assessment (90 days)	proqol.org		N/A — bring results to supervision
Additional module: _____			☐ Yes ☐ No
Additional module: _____			☐ Yes ☐ No



Staff Name: _____ Staff Signature / Date: _____



Appendix E. KEY CONTACTS AND EMERGENCY NUMBERS

Keep this page accessible at all times. Post it near your workstation.

Contact	Number / Information
Emergency — Police / Fire / Ambulance	911
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
Advocate for Children and Youth	1-800-476-3933
Helpline for Children	1-800-663-9122
BC Crisis Centre (24/7)	1-800-784-2433
Kids Help Phone (24/7)	1-800-668-6868 or text HOME to 686868
Poison Control	1-800-567-8911
WorkSafeBC (to report serious incident)	1-888-621-7233
EFAP (24/7)	[Posted in staff area]
On-call Supervisor	[Current schedule posted weekly in staff area]
Program Manager	[Name and direct number — posted in staff area]
Executive Director	[Name and number — posted in staff area]
Youth's MCFD Social Worker(s)	[Posted in ISR for each youth; see Program Manager]
Facility Address	[Your facility address — for emergency callers]
HealthLink BC (nurse line, 24/7)	8-1-1

Appendix F. GLOSSARY OF KEY TERMS

You will encounter these terms regularly. Know what they mean.

Term	Definition
ASIST	Applied Suicide Intervention Skills Training — intensive 2-day workshop on suicide intervention; required within 60 days
BSP	Behaviour Support Plan — individualized document identifying triggers, de-escalation strategies, and approved interventions for a specific youth
CCALA	Community Care and Assisted Living Act — BC legislation governing licensing of community care facilities including group homes
CFCSA	Child, Family and Community Service Act — BC's primary child welfare legislation; governs rights, protection, and placement of children in care
CI	Critical Incident — any event requiring mandatory reporting to MCFD; see Part 6 for categories and timelines
CLBC	Community Living British Columbia — Crown agency funding and arranging supports for adults with developmental disabilities
DAA	Delegated Aboriginal Agency — Indigenous-governed organization that has been delegated authority to provide child and family services to Indigenous communities
DRIPA	Declaration on the Rights of Indigenous Peoples Act — BC legislation implementing UNDRIP; shapes Indigenous child welfare
EFAP	Employee and Family Assistance Program — confidential counselling and support available to all staff from Day 1
FOIPPA	Freedom of Information and Protection of Privacy Act — governs how personal information is collected, used, and disclosed
ISP	Individual Service Plan — the central care planning document for each youth; governs all aspects of their care
ISR	Individual Service Record — the complete file for each youth, including ISP, health records, shift logs, and all documentation
MAR	Medication Administration Record — the legal record of all medications administered to a youth
MCFD	Ministry of Children and Family Development — BC ministry responsible for child protection and residential care
NVCI	Non-Violent Crisis Intervention — certification in verbal and physical de-



	escalation techniques; required within 90 days
OFA	Occupational First Aid — workplace first aid certification; minimum Level 2 required per shift
OHS	Occupational Health and Safety — workplace safety system governed by the Workers Compensation Act
PCS	Provincial Centralized Screening — MCFD's 24-hour intake and reporting line: 1-800-663-9122
RCY	Representative for Children and Youth — independent officer of the BC Legislature who advocates for children and youth in care
RCR	Residential Care Regulation — BC regulation setting operational and physical standards for licensed residential care facilities
s.14 CFCSA	Section 14 — the mandatory reporting provision; requires every person to report suspected child protection concerns immediately
s.70 CFCSA	Section 70 — the rights of children and youth in care; you are required to actively uphold every right listed
STS	Secondary Traumatic Stress — occupational health impact of regular exposure to others' trauma; a recognized risk of this work
UNDRIP	UN Declaration on the Rights of Indigenous Peoples — international instrument whose rights DRIPA requires BC to implement
WHMIS	Workplace Hazardous Materials Information System — Canada's national system for labeling and communicating chemical hazards